

Question:

A baby is born after the 32nd gestational week by cesarean delivery. The mother suffered from gestational diabetes; however, she had no other pregnancy-related diseases and was otherwise healthy. The baby has a blood pressure of 100/58 mm Hg, heart rate of 104/min, and oxygen saturation of 88%. The child has tachypnea, subcostal and intercostal retractions, nasal flaring, and cyanosis. The cyanosis is responding well to initial administration of oxygen. The nasogastric tube was positioned without problems. Which of the following is the most likely diagnosis?

Answer Choices:

- A. Neonatal respiratory distress syndrome (NRDS)
- B. Tracheoesophageal fistula
- C. Pneumonia
- D. Congenital heart anomaly with right-to-left shunt

Reasoning + Answer:

<think> Assessing the symptoms described, we need to consider the cause of the newborn's respiratory distress. The presence of tachypnea, subcostal and intercostal retractions, and nasal flaring indicate respiratory distress. The cyanosis, which is a sign of inadequate oxygenation, and its response to oxygen administration also suggest respiratory distress. These symptoms are classic signs of a condition that affects the lungs or airways. The presence of cyanosis as an immediate issue, and its quick response to oxygen implies a condition most closely related to a respiratory issue and possibly affecting lung development, given the birth week and symptoms described. Neonatal respiratory distress syndrome typically presents in preterm infants but considering the provided symptoms this condition becomes a well-fitting explanation.</think><answer>A. Neonatal respiratory distress syndrome (NRDS)</answer>

